CLINICAL TRIAL FAILURES · MISSED ENDPOINTS · IMMUNOLOGY & AUTOIMMUNE PHASE 2,3 · STARTS 2015–2024

# FcRn: 4 of 4 completed trials missed their primary endpoint

**4 completed trials across 3 sponsors and 3 molecules ran to the end and missed their primary endpoint on the sponsor's own posted result; 0 met it.** Only 1 of 19 closed trials were stopped early for efficacy or safety, which is why this class is read through its completed trials rather than a stop rate. Most completed trials post nothing readable, so the count is a floor.

**4 of 4**
completed trials with a readable primary result missed it · 0 met it

**3 / 3**
sponsors / molecules behind the misses

**5.3%**
stopped early: 1 of 19 closed trials (95% CI 0.9%–24.6%)

**67.9%**
of 28 trials have closed · 9 still open or unresolved

**4 of 4** **Completed and missed the primary endpoint.** Of 12 completed trials in this segment, 4 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 4 missed and 0 met. Counted separately and never in the rate above; a floor, not a rate. 3 of the 9 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

## The completed trials that missed

| Trial | Ph | Sponsor | Experimental drugs | Started | What the posted result says |
|---|---|---|---|---|---|
| NCT06028438 | 2 | Johnson & Johnson | NIPOCALIMAB | 2023-08 | Change From Baseline in Disease Activity Index Score 28 Using C-reactive Protein (DAS28-CRP) at Week 12 — p 0.822, no threshold stated; 0.2 or above, so not significant under any conventional threshold. Least square mean difference -0.06 (95% CI -0.6 to 0.48). Method: ANCOVA. |
| NCT04991753 | 2 | Johnson & Johnson | NIPOCALIMAB | 2021-10 | Change From Baseline in Disease Activity Index Score 28 Using C-reactive Protein (DAS28-CRP) at Week 12 — p 0.224, no threshold stated; 0.2 or above, so not significant under any conventional threshold. Least square mean difference -0.45 (95% CI -1.17 to 0.28). Method: ANCOVA. |
| NCT04687072 | 3 | argenx | EFGARTIGIMOD ALFA | 2020-12 | Percentage of Participants With Chronic Immune Thrombocytopenia (ITP) With a Sustained Platelet Count Response Between Weeks 19 and 24 — p 0.5081, no threshold stated; 0.1 or above, so not significant under any conventional threshold. Difference in percentage -3.5 (95% CI -14.7 to 7). Method: Cochran-Mantel-Haenszel. |
| NCT03861481 | 2 | UCB Biopharma S.P.R.L. | ROZANOLIXIZUMAB | 2019-03 | Change From Baseline to Week 13 (Day 85) in Inflammatory Rasch-built Overall Disability Scale (iRODS) Score — LS Mean difference (Rozimab - Placebo) -0.052 (90% CI -0.892 to 0.788), no p-value posted and no threshold stated: the interval is consistent with no effect under any conventional threshold, so not significant. |

## How this compares

FcRn



5.3%

1/19 closed

all immunology & autoimmune trials with a resolved drug target



3.5%

30/865 closed

All immunology & autoimmune Phase 2,3



3.4%

68/1989 closed

**What is in this class:** 5 distinct experimental drugs across 28 trials (antibody 18, protein 10, unknown 1). Most tested: EFGARTIGIMOD ALFA (10), ROZANOLIXIZUMAB (5), NIPOCALIMAB (5), BATOCLIMAB (5), ORILANOLIMAB (3). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

**How much does this rest on one decision?** The one stop came from 1 sponsor-asset programme across 1 sponsor; the largest (Immunovant Sciences GmbH) contributed 1; removing that programme's trials from both sides leaves 0.0%. Screened alongside 28 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. A further 2 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 15.8%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

## By start cohort

| Trials started | Stops | Closed | Rate | 95% CI |
|---|---|---|---|---|
| 2018–2020 | 0 | 9 | 0.0% | 0.0%–29.9% |
| 2021–2024 | 1 | 9 | 11.1% | 2.0%–43.5% |

## Where the stops sit

Efficacy / futility: 1.
Sponsors with most stops: Immunovant Sciences GmbH (1).

## The stopped trials

| Trial | Ph | Sponsor | Experimental drugs | Stopped | Type | Registry stop reason |
|---|---|---|---|---|---|---|
| NCT05517447 | 3 | Immunovant Sciences GmbH | BATOCLIMAB | 2026-04 | Efficacy | The feeder studies (i.e., IMVT-1401-3201 and 3202) for study IMVT-1401-3203 failed to achieve their pre-defined primary efficacy outcomes. Accordingly, there was a change to the benefit-risk balance. No new safety findings were observed. |

**Method**
Denominator: ClinicalTrials.gov interventional Phase 2,3 immunology & autoimmune trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 48% of industry immunology & autoimmune trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

**Limits**
Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 67.9% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

**The whole cohort, trial by trial.** The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. €99, delivered immediately: **clinicaltrialfailures.com/asset-check**

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.